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NOTE

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Factoring and Self-Referral: Limits and Opportunities

This newsletter is for informational purposes only and is NOT legal advice. For legal advice consult a lawyer.

Over the last few years in Pennsylvania, we have advised client physicians regarding the legitimacy of factoring claims for supplies they have ordered themselves. By that we mean, the physician or a company he owns for this purpose, buys the insurance claim from the entity from which he has ordered a supply, and then submits the claim, usually to a Workers' Compensation insurer. Here, this activity has been conducted in the face of a Workers' Compensation law provision against self-referral. In a recent Supreme Court of Pennsylvania decision, the court reopened the door to this activity which had to be stopped in light of a Commonwealth Court case. The approach may be useful to physicians in other states, but would be prohibited under some state laws. We explain.

The Pennsylvania Story

The Pennsylvania Workers' Compensation statute has an anti-self referral provision, which says physicians may not refer a patient to a provider of "laboratory, physical therapy, rehabilitation, chiropractic, radiation oncology, psychometric, home infusion therapy or diagnostic imaging, goods or services" if they have a financial interest with the entity receiving the referral. Rather than write their own regulations interpreting what "a financial interest with" might mean, they issued a regulation that said they would accept anything that would be compliant with

Stark exceptions or anti-kickback safe harbors. In addition, in 2014, the state legislature adopted a provision essentially prohibiting physicians from dispensing drugs to Workers' Compensation patients.

In 2023, a pharmacy owned in part by physicians who referred to it, applied for Workers' Compensation payment which was denied by the insurer on the grounds of the self-referral prohibition. The pharmacy appealed and the hearing officer held that the phrase "goods and services" included pharmacy and drugs in the ambit of the anti-referral provision. The pharmacy appealed to the

Commonwealth Court, an intermediate court in Pennsylvania that considers issues of statutory interpretation. Not only did they affirm that “goods and services” included pharmacy, they also over-reached regarding the Stark application and imputed into the PA statute Stark’s list of designated health services, which includes outpatient drugs.

The Supreme Court, in a 5-2 decision, in the *700 Pharmacy* case, brought common sense back. They said the specifically enumerated services were all that the prohibition applies to and that if the legislature had wanted to include pharmacy and drugs it could have, as it did in other sections of the statute. They also reverted the applicable law to permit referrals that comply with Stark exceptions or antikickback safe harbors, refuting that the regulators intended to incorporate the statutory provisions under either federal law.

Factoring

In this model, the referring physician buys the claim for Worker’s Compensation payment from the provider to which he has referred, and then submits the claim to be paid on his own number. In some states, this is prohibited. It is not prohibited in Pennsylvania, although no court has opined on it, either. The provider who sells the claim gets a discounted rate from the physician or his entity, without having to do any insurance billing with all of its vagaries and pitfalls. The physician takes the risk of the insurer paying by submitting the claim showing properly the actual provider who supplied the goods but the physician or his related entity as the payee.

This CANNOT work for federal claims, which do not permit factoring under rules literally referred to as the anti-factoring rules pertaining to how claims may be assigned from one party to another. They are also known as the reassignment rules. It also cannot work for laboratory services because of the Eliminating Kickbacks in Recovery Act (EKRA), which has its own list of prohibitions in referring to laboratories pursuant to financial relationships and goes far beyond federal payments to any public or private insurance programs. Although enacted in 2018, while a couple of court cases have interpreted its application, there are still no regulations under it, despite the Secretary’s clear authority to issue them.

Significance

For those physicians in Pennsylvania who ceased factoring Workers’ Compensation pharmacy claims after the Commonwealth Court opinion, they can go back to what they were doing. For physicians who were wary of even trying this model, it is now worth reconsidering.

For physicians in other states, it may be worth considering whether this type of approach is feasible under your state’s laws. Anyone who would like to speak to us further about potential opportunities should feel free to get in touch.